**INDICATION: Bilateral primary open-angle glaucoma (POAG) with progression of visual field loss and sub-target intraocular pressures on dual topical therapy (latanoprost and dorzolamide/ timolol).**

**PROCEDURE PERFORMED: Selective Laser Trabeculoplasty (SLT), right eye.**

**PROCEDURAL DETAILS: Pre-procedure intraocular pressure (IOP) in the right eye measured 24 mmHg by Goldmann applanation tonometry. One drop of apraclonidine 0.5% was instilled in the right eye to prevent post-laser IOP spikes, followed by topical proparacaine 0.5% for corneal anesthesia.**

The patient was seated at the Lumenis Selecta II SLT machine with forehead firmly against the rest. A Latina single-mirror SLT gonioscopy contact lens with methylcellulose coupling fluid was positioned onto the right cornea. The anterior chamber angle was evaluated: 360-degree view demonstrated wide open angles (Shaffer Grade 4) with moderate, uniform trabecular meshwork pigmentation without peripheral anterior synechiae or angle recession.

The Q-switched frequency-doubled Nd:YAG laser (wavelength 532 nm, spot size 400 microns, pulse duration 3 nanoseconds) was focused on the pigmented trabecular meshwork. Energy was titrated initially to 0.8 mJ, observing for the appearance of minimal champagne-like microbubble formation, and adjusted to 0.7 mJ. A total of 52 non-overlapping laser applications were systematically placed over 180 degrees of the inferior and nasal trabecular meshwork.

The contact lens was removed, and the cornea rinsed with sterile balanced salt solution. Post-procedure drop of apraclonidine 0.5% was instilled. Intraocular pressure checked at one hour post-procedure measured 18 mmHg (no spike). The patient was discharged with a prescription for topical prednisolone acetate 1% QID for five days and instructed to maintain baseline glaucoma medications.